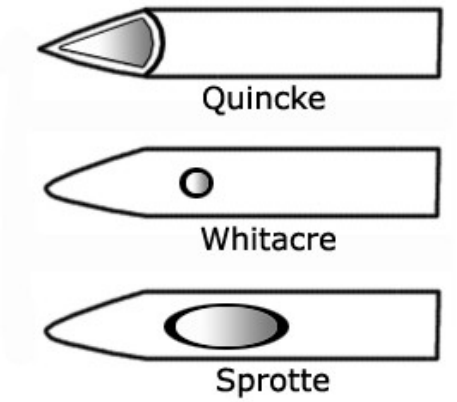
Spinal needles have evolved over the past decades into several forms. Three of the major forms (Quincke, Whitacre, and Sprotte) are pictured to the right. The primary difference noticed between the Quincke and its two counterparts are that it has a cutting tip while the latter both have blunt, non-cutting tips. The primary difference between the Whitacre and Sprotte needles are that the injection port on the Sprotte is larger than that of the Whitacre. All three needles contain a close- fitting introducer needle designed to give a reinforcing strength to the needle and prevent the injection port from collecting tissue from the epidermis and deeper tissue structures and depositing them in the spinal compartment.



Due to considerable research demonstrating that the blunt-tipped (also referred to as pencil-point) needles have a lower incidence of post-dural puncture headache following the administration of a spinal anesthetic, the current practice is to use blunt-tipped needles.

It should also be noted that research demonstrates that the lower-cost Whitacre needle has no appreciable difference in the risk of PDPH than its more expensive cousin, the Sprotte needle.

Wong, CA. Spinal and Epidural Anesthesia. New York: McGraw-Hill; 2007: 33. Campbell DC, et al. (1993). Comparison of the 25-gauge Whitacre with the 24-gauge Sprotte spinal needle for elective Caesarean section: cost implications. Canadian Journal of Anesthesia, 40(12), 1131-1135.

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